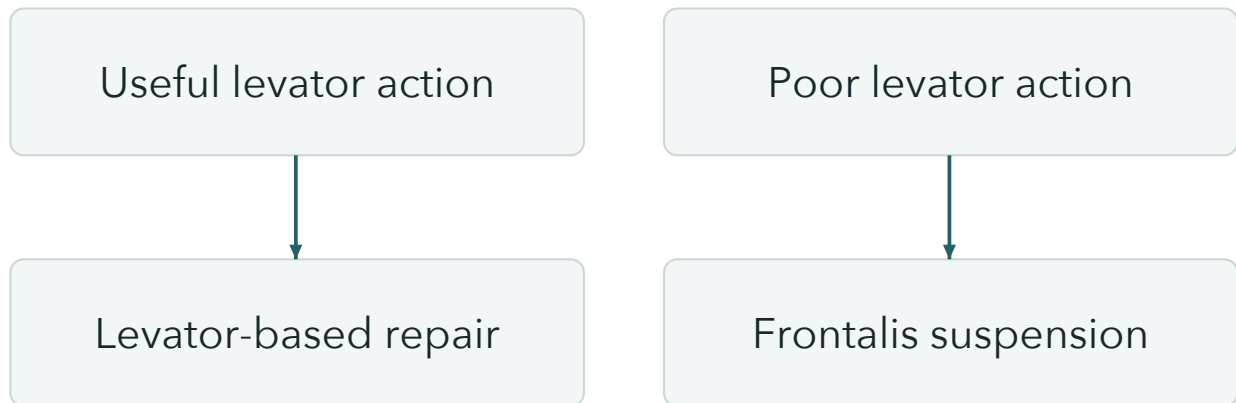


Ptosis lid malposition and reconstruction

Part 2 | Ptosis: measure the muscle before choosing the operation

Lid height alone cannot select a repair.



Measure with the brow fixed. Corneal protection constrains the target lid height.

Original decision sketch: useful levator action versus poor levator action.

Measure excursion with the brow fixed and assess pupil, motility, fatigue, lid crease and surface protection. Poor levator function can favour a frontalis suspension, whereas useful function permits levator-based approaches in suitable cases. A fluctuating or neurological pattern needs its cause addressed. Explain the risk of lagophthalmos and the intended postoperative height, not merely a cosmetic correction.

Involucional entropion: identify each component

Repeat temporary rotation may leave the underlying laxity.

Finding	Implication
Horizontal laxity	Delayed snap-back and excessive distraction
Retractor dysfunction	Reduced lower-lid movement in downgaze
Orbicularis override	Additional inward rotational force
Definitive repair	Address the demonstrated components

Select a correction based on the examination rather than the diagnostic label alone. A recurrence after everting sutures can reveal persistent horizontal laxity or retractor failure. Distinguish this from a scar-shortened posterior lamella that needs a different reconstruction. The oral answer should name which force the operation changes and which recurrence mechanism remains possible.

Ectropion and cicatricial shortage

Tightening alone cannot replace missing skin.

Finding	Implication
Lax involutional lid	Consider horizontal support correction
Anterior-lamellar scar shortage	Restore tissue length when indicated
Medial punctal displacement	Assess the drainage and lid component
Facial weakness	Protect the surface and review closure

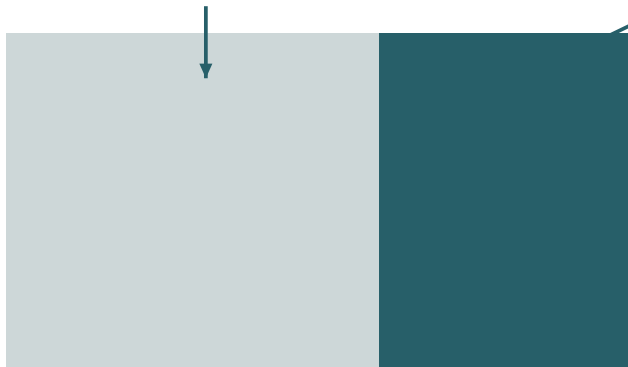
Assess anterior and posterior tissue length, canthal support and blink before choosing a procedure. A graft or flap addresses shortage, while a tightening operation addresses laxity; some eyes need both. Tear overflow may persist if the punctum or pump remains dysfunctional. Explain how the reconstruction protects the cornea as well as repositioning the margin.

Full-thickness reconstruction

At least one lamella must provide vascular support.

Anterior lamella
Skin + orbicularis

Posterior lamella
Tarsus + conjunctiva



A free graft requires a vascularised bed.

Original section schematic: restore lining, support, movement and margin continuity.

Original lid section: skin-orbicularis anteriorly, tarsus-conjunctiva posteriorly.

Restore lining, support, margin continuity and movement. A free graft needs a vascularised bed; opposing free grafts alone risk inadequate survival. Choose the combination according to defect size, site and remaining tissue rather than memorising one flap name. Consider lash contact, notching, closure and donor-site consequences when explaining the expected functional result.

Postoperative review: the cornea is an outcome

An anatomically raised lid can still be an unsafe result.

Finding	Implication
Excess height or poor closure	Assess exposure and epithelial injury
Residual malposition	Identify remaining mechanical component
Margin irregularity	Check contact with the ocular surface
Child with ptosis	Continue optical and amblyopia assessment

Review comfort, blink, closure, tear film and visual function as well as symmetry. The first response to exposure is protection and assessment of severity; revision may be needed if the mechanism persists. Discuss the possibility of staged adjustment and recurrence in consent. A successful photograph is not sufficient evidence of a safe functional repair.